

Clinician-Driven Model for Adapting Mental Health Interventions for Neurodivergent Youth
 Delage et al. 2026 Seattle FASD Conference

Step	Practice Phase	Key Actions and Embedded Community Engagement
1	Begin with Listening and Relationship-Building	Engage teens and caregivers in open conversations about what has and has not worked in prior treatment. Ask directly about barriers to understanding, pacing, generalization, and emotional safety. Treat families as experts in the teen's neurodevelopmental profile and incorporate their input from the outset.
2	Identify Accessibility Barriers in the Existing Intervention	Review current materials and delivery structures for complexity, cognitive load, abstract language, and pacing demands. Ask families for feedback on what feels confusing, overwhelming, or inaccessible. Observe patterns of misunderstanding without attributing them to lack of motivation.
3	Preserve Core Treatment Elements	Identify which components of the intervention are essential to theoretical integrity (e.g., skills modules, behavioral hierarchy, consultation structures). Avoid removing core mechanisms of change. Community feedback should help clarify which barriers relate to delivery versus content.
4	Modify Delivery for Neurodevelopmental Fit	Simplify language, increase repetition, use visual supports, reduce abstract metaphors, and build in structured rehearsal. Increase caregiver integration to support generalization across settings. Collaborate with families to test whether modifications improve comprehension and real-world implementation.
5	Adjust Expectations and Measurement of Progress	Recognize that observable behavioral change may occur more slowly and inconsistently. Distinguish between skill knowledge and skill implementation. Invite caregivers to help define meaningful indicators of progress within home and school contexts.

6	Build Peer and Community Connection When Possible	Facilitate opportunities for connection among families navigating similar challenges, when clinically appropriate. Normalize shared experiences and reduce isolation. Seek informal consultation with regional FASD specialists or professional networks when available.
7	Engage in Ongoing Reflection and Consultation	Regularly review what is working and what is not with families. Seek supervision or peer consultation when possible. Adapt iteratively while maintaining core treatment fidelity. Treat adaptation as an ongoing collaborative process rather than a one-time modification.
